

CJC-1295 / Ipamorelin

Patient Education Handout

Functional Health & Longevity Program · KORB Health Group · For Patient Reference

This handout is for educational reference only. It is not medical advice. Always follow your prescription label, your pharmacy-specific instructions, and your KORB provider's guidance. If you have questions, contact KORB by phone or email (see contact information below).

What Is CJC-1295 / Ipamorelin

CJC-1295 / Ipamorelin is a dual-peptide combination used to stimulate growth hormone (GH) release. CJC-1295 extends GH-releasing signals while Ipamorelin directly stimulates GH release from the pituitary gland. Together they promote a more sustained, pulsatile GH effect.

Like Sermorelin, this combination stimulates your own pituitary to produce GH — it does not replace growth hormone. It acts through your body's existing system.

Your IGF-1 level is one of the key response markers your provider will monitor throughout your program.

Possible Support From CJC-1295 / Ipamorelin

This medication is one possible adjunctive support option for patients with goals related to:

- Body composition — lean mass preservation and fat metabolism support as part of a lifestyle program
- Sleep quality and recovery physiology
- GH/IGF-1 axis support
- Exercise recovery and general wellness

Response varies from patient to patient. CJC-1295/Ipamorelin has human pharmacokinetic data, but body composition benefits are extrapolated. Your provider will assess your IGF-1 and response at your follow-up visit.

How to Use CJC-1295 / Ipamorelin

Your prescription label — or the instructions given to you by your KORB clinical provider — is the authoritative source for your dose and directions. (Some pharmacies label vials “as directed by provider” rather than printing exact instructions on the label.) What follows is a general reference. **Do not adjust anything without speaking to your provider first.**

How to inject	When to inject	Schedule	Active weeks	Off weeks
Subcutaneous (SQ) injection — fatty tissue under the skin	At bedtime — on an empty stomach, at least 2 hours after eating	6 days ON, 1 day OFF — Monday–Saturday; Sunday off	The first 12 weeks of your 16-week cycle	A 4-week washout before your next cycle begins

Timing Matters

Inject at bedtime on an empty stomach. GH is released naturally during sleep. Food — particularly carbohydrates — can blunt GH release. Wait at least 2 hours after your last meal. Sunday is your rest day every week — do not inject on Sundays.

Your dose is compounded specifically for you. Always follow the units and schedule on your prescription label — do not estimate or adjust based on a prior fill.

Missed a Dose?

- If you miss a dose on a scheduled day: skip it — do not double up the next day

Traveling with Your Medication

Traveling doesn't have to interrupt your program. Keep your medication with you — in your carry-on or personal item, not checked baggage — in its original labeled vial or packaging. It doesn't need to be packed in ice or kept refrigerated during travel; just keep it away from excessive heat (above 86°F / 30°C) and from freezing. Once you reach your destination, refrigerate it again. If flying, medication and syringes are permitted in carry-on luggage; TSA recommends keeping the original pharmacy label visible. Traveling across time zones can shift your injection time by a few hours without a problem — if you'll be off your usual schedule for several days, contact KORB for guidance.

Storage and Handling

Refrigerate 36°F – 46°F (2°C – 8°C)	Do not freeze Freezing damages medication	Protect from light Keep away from direct sunlight	Discard at 28 days Write open date on vial when opened
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Before first use: refrigerate, protect from light, do not freeze or shake. After opening: write the open date on your vial and discard 28 days after first use — even if medication remains. Do not use if the solution appears cloudy, discolored, or contains particles.

What to Expect — Realistic Timeline

Timeline	Phase	What to Expect
Weeks 1–4	Starting out	Some patients notice early improvements in sleep or morning energy. Many notice nothing yet — this is normal.
Weeks 4–8	Early response	Sleep, recovery, and body composition changes may begin to emerge. Do not adjust your schedule.
Weeks 8–12	Continued support	Ongoing GH/IGF-1 axis support. Your provider will review IGF-1 at your follow-up visit.
Weeks 13–16	Off Cycle — Labs & Lifestyle Focus	No injections — this is intentional. Your labs (drawn between weeks 12 and 14 of your cycle) are reviewed during this window, and this is a good stretch to focus on nutrition, exercise, sleep, and other lifestyle habits before your next cycle. Do not restart early without provider direction.

Do not compare your response to others. Response is highly individual and influenced by age, baseline GH levels, body composition, sleep quality, and lifestyle habits.

Side Effects and What to Watch For

What You May Notice — Common	What to Do
Injection site redness or irritation	Common and usually mild. Rotate injection sites. Tell KORB provider if persistent.
Flushing or warmth after injection	Typically brief. Usually resolves within minutes.
Headache	May occur early in therapy. Usually mild and short-lived.
Fatigue	Uncommon; tell KORB provider if persistent.

Worth Monitoring — Tell KORB Provider at Your Next Visit

Water retention or puffiness	Can occur with GH stimulation. Tell KORB provider if noticeable.
Joint discomfort or tingling	May indicate dose adjustment is needed. Tell KORB provider.
Glucose changes	GH stimulation can affect blood sugar. Tell KORB provider if you notice changes.

Stop injecting and seek emergency care immediately if you experience: trouble breathing, swelling of the face, lips, tongue, or throat, severe rash or hives, chest pain, or any symptom that feels severe or unsafe.

Lab Monitoring

Lab monitoring is a required part of your program — not optional. Your provider uses labs to assess your response and safety.

- IGF-1 — primary response marker. Too high is not better; target physiologic range.
- HbA1c and fasting glucose — GH stimulation can affect glucose metabolism.
- Thyroid (TSH, free T4, free T3) — a functioning thyroid axis is important for optimal GH response.
- CBC, CMP, lipid panel — standard panel at baseline and every 16-week follow-up.
- PSA — added for men aged 45 and older at every draw.

Your lab draw should be completed between weeks 12 and 14 of your active cycle, so results are back in time for your 16-week follow-up visit — some results, like IGF-1, can take about a week to come back. Unless your lab order says otherwise, plan to fast overnight and have your blood drawn first thing in the morning.

Safety Reminders

- Use CJC-1295/Ipamorelin only as prescribed. Do not change dose, schedule, or route without provider direction.
- Do not combine with other GH-axis therapies unless specifically instructed by KORB.
- CJC-1295/Ipamorelin is not appropriate during active malignancy, uncontrolled thyroid disease, or pregnancy.
- Do not restart after your off weeks early — the washout period is intentional.
- Tell KORB provider if you develop any new medical condition, start a new medication, or become pregnant.
- Your prescription label is the authoritative source for your dose and instructions.
- Use a new insulin needle with syringe for every injection — do not reuse.
- Never mix peptides — use one insulin needle with syringe for every peptide, every injection. Do not combine into one injection or syringe.
- Follow your state or local regulations for sharps disposal — use an FDA-cleared sharps container; do not place used needles or syringes in household trash unless your local guidelines specifically allow it.

When to Contact KORB and When to Seek Emergency Care

Contact KORB Operations

- Questions about timing, storage, or administration

Email: info@korbhealth.com

Phone: +1 (888) 959-7299

Hours: Mon–Fri, 9 AM–6 PM CT

Patient Portal — Message Your Provider

- Injection-site irritation or redness that does not resolve
- Mild but persistent side effects
- Questions about whether to continue therapy

Seek Emergency Care Immediately

- Trouble breathing or throat / face swelling
- Severe rash or widespread hives
- Chest pain or fainting
- Severe or rapidly worsening symptoms
- Confusion or severe weakness
- Any symptom that feels urgent or unsafe

The Patient Portal is your only HIPAA-compliant way to message your provider directly — providers check portal messages once per day on their clinic days. For anything that doesn't need your provider's direct medical judgment, contact KORB Operations by phone or email instead.

Phone and email are not appropriate for emergencies. When in doubt, go to urgent care or the emergency room.

Key Reminders

CJC-1295/Ipamorelin stimulates your own GH — it is not GH replacement.

Inject at bedtime, on an empty stomach, Monday through Saturday. Sunday off.

Always follow your prescription label for your exact dose and units.

Write the open date on your vial — discard 28 days after first use.

Labs between weeks 12–14 are required before your 16-week follow-up visit.

Contact KORB Operations by phone or email for non-urgent questions, or your provider via the Patient Portal for medical questions. Seek emergency care for severe symptoms.